

# COOK COUNTY CHILDREN'S HOSPITAL

Department of Pediatric Hospital Medicine

## DISCHARGE SUMMARY

|                      |                                                                |                       |                                                       |
|----------------------|----------------------------------------------------------------|-----------------------|-------------------------------------------------------|
| <b>PATIENT NAME</b>  | Marcus D. Whitfield                                            | <b>ADMISSION DATE</b> | 02/26/2026                                            |
| <b>DATE OF BIRTH</b> | 03/03/2016                                                     | <b>DISCHARGE DATE</b> | 03/02/2026                                            |
| <b>AGE</b>           | 9 years, 11 months                                             | <b>LENGTH OF STAY</b> | 5 days                                                |
| <b>MRN</b>           | PED-2025-04471                                                 | <b>ATTENDING</b>      | Heather Collins, MD   Hospital Pediatrics   IL-315904 |
| <b>SEX</b>           | Male                                                           | <b>DISCHARGING MD</b> | Heather Collins, MD                                   |
| <b>INSURANCE</b>     | Medicaid — DCFS Foster Care (Illinois)   Member ID: 672481905A | <b>DISPOSITION</b>    | Home with licensed foster parent (Ms. Renee Carter)   |

**Consulting Services:** Pediatric Critical Care Medicine, Respiratory Therapy, Pediatric Nutrition

**Caregiver Contact:** Ms. Renee Carter (312) 555-0184

### REASON FOR ADMISSION

Acute severe asthma exacerbation with respiratory failure requiring endotracheal intubation and mechanical ventilation. This represents the patient's fourth lifetime intubation for asthma.

### HISTORY OF PRESENT ILLNESS

Marcus is a 9-year-old male with a well-established history of poorly controlled, severe persistent asthma who was brought to the Emergency Department on 02/26/2026 by his foster mother, Ms. Renee Carter, after approximately 18 hours of progressive respiratory distress unresponsive to home nebulizer treatments and albuterol MDI. Ms. Carter reported that Marcus had developed increased cough, wheeze, and work of breathing the evening prior, and that he had been using his albuterol inhaler multiple times without meaningful relief. She administered two albuterol nebulizer treatments at home prior to arrival.

On presentation to the Emergency Department, Marcus was in moderate-to-severe respiratory distress with accessory muscle use, nasal flaring, subcostal retractions, prolonged expiratory phase, and diffuse expiratory wheeze with diminished air entry bilaterally. He was unable to speak in full sentences. Initial oxygen saturation on room air was 84%.

Given failure to respond to aggressive bronchodilator therapy — including continuous albuterol nebulization, ipratropium bromide, systemic corticosteroids, and intravenous magnesium sulfate (50 mg/kg over 20 minutes) — the decision was made to proceed with endotracheal intubation. The patient was intubated without complication by the Pediatric Critical Care team and admitted to the Pediatric Intensive Care Unit.

This is the fourth episode in Marcus's life requiring intubation for asthma and the first such episode in the current foster placement. Prior intubations are documented in hospitalization records from 2022, 2023, and 2024.

### PAST MEDICAL HISTORY

- Asthma, severe persistent — poorly controlled; 4 lifetime intubations (including current)
- Autism Spectrum Disorder, Level 2
- ADHD, Combined Type
- Post-traumatic Stress Disorder
- Fetal Alcohol Spectrum Disorder
- Suspected inflammatory bowel disease (Crohn's disease), histologic confirmation pending
- Pica with elevated blood lead level (most recent value 18 mcg/dL, 01/22/2026)
- Neurofibromatosis Type 1
- Iron deficiency anemia

**Past Surgical History:** None documented

**Allergies:** NKDA per caregiver report

### SOCIAL HISTORY

Marcus resides with licensed foster parent Ms. Renee Carter at 1847 S. Hamlin Ave., Chicago, IL 60623. He is currently enrolled in 4th grade at Lincoln Elementary School, approximately 6 weeks into this school placement. Biological mother is not currently contactable. There is no known biological father of record. DCFS caseworker assigned.

Tobacco/smoke exposure: Unknown; not assessed during this hospitalization. Prior records do not document confirmed household smoke exposure in current placement.

## MEDICATIONS ON ADMISSION

Per foster parent and handwritten medication list provided at admission:

1. Budesonide/Formoterol (Symbicort) 80/4.5 mcg (2 puffs BID)
2. Albuterol MDI 90 mcg/puff (2 puffs q4-6h PRN)
3. Albuterol 2.5 mg/3 mL nebulizer solution (q4h PRN)
4. Risperidone 1.5 mg QHS
5. Methylphenidate ER 27 mg QAM
6. Clonidine 0.1 mg QHS
7. Montelukast 5 mg QHS (listed; adherence uncertain per caregiver)
8. Ferrous sulfate 325 mg daily
9. Melatonin 3 mg QHS (OTC)
10. PEG 3350 17 g daily (OTC)

## HOSPITAL COURSE

### Days 1-2 (02/26-02/27/2026): PICU

Marcus was managed on pressure-controlled ventilation following intubation. Ventilator settings were titrated by the Pediatric Critical Care team. He received continuous albuterol nebulization via in-line circuit, intravenous methylprednisolone (1 mg/kg/dose q6h), and inhaled ipratropium. Sedation was maintained with intermittent midazolam and fentanyl per Critical Care protocol. Chest radiograph obtained on 02/26/2026 demonstrated a right upper lobe infiltrate with associated hilar fullness. This was interpreted in the clinical context as consistent with an infectious or inflammatory process, most likely atypical pneumonia in the setting of an acute asthma exacerbation. Azithromycin was initiated empirically for atypical pneumonia coverage (10 mg/kg on day 1, 5 mg/kg days 2-5). Psychiatric medications were held during the ICU phase due to nil per os status and sedation requirements; the clinical pharmacist confirmed that a brief hold of risperidone and clonidine over 48-72 hours was acceptable given the clinical circumstances. Methylphenidate was held throughout the hospitalization.

### Day 3 (02/28/2026): Extubation and Step-Down

Marcus was successfully extubated on the morning of 02/28/2026 following demonstration of adequate respiratory effort and ventilatory mechanics. He was transitioned to a high-flow nasal cannula at 6 L/min and then weaned to low-flow oxygen by end of day. He was transferred from the PICU to the general pediatric ward. Post-extubation, Marcus was noted to be behaviorally dysregulated, which was attributed to the ICU environment, sedation effects, and baseline neurodevelopmental vulnerabilities. He required one-on-one nursing observation for a portion of day 3. Child Life was consulted informally to provide sensory-calming support. Respiratory Therapy continued scheduled albuterol treatments and initiated spacer training for caregiver. Pediatric Nutrition consulted due to known iron deficiency anemia and concern for inadequate caloric intake in the setting of suspected IBD. Ferrous sulfate was continued per prior outpatient regimen during this admission. Nutritionist documented that protein and caloric intake remained below estimated needs, and recommended high-calorie oral supplementation (PediaSure 1.5 kcal/mL, 1 can BID) as a bridge measure, with formal outpatient dietary planning to follow.

### Days 4-5 (03/01-03/02/2026): Continued Monitoring and Discharge Preparation

Marcus was weaned to room air without supplemental oxygen by 03/01/2026 and maintained saturations of 97-99% on room air through 03/02/2026. Wheeze largely resolved. Oral intake improved. He was tolerating his home medications by mouth. Inhaled corticosteroid regimen was stepped up per NAEPP guidelines given the severity of this exacerbation and the prior history of multiple intubations. He was discharged on Symbicort 160/4.5 mcg 2 puffs BID in lieu of the prior 80/4.5 mcg formulation. Oral prednisolone taper was prescribed at discharge (see discharge medications below). Azithromycin course to be completed as outpatient.

## RADIOLOGY

### Chest Radiograph — 02/26/2026

Ordered by: ED Team / Read by: Radiology

Portable AP chest radiograph obtained at time of admission. Endotracheal tube noted in appropriate position. The lungs demonstrate right upper lobe infiltrate with associated hilar fullness/lymphadenopathy. The remainder of the lung fields shows hyperinflation with flattening of the hemidiaphragms consistent with the clinical diagnosis of acute severe asthma. No pneumothorax. No pleural effusion.

Impression: Right upper lobe airspace opacity — likely infectious/inflammatory in etiology, consistent with atypical pneumonia in the appropriate clinical context. Hilar prominence noted. Outpatient follow-up of this radiographic finding is recommended; correlation with

clinical history advised if further workup is deemed indicated.

## DISCHARGE DIAGNOSES

1. Acute severe asthma exacerbation with respiratory failure — 4th intubation
2. Atypical pneumonia (suspected *Mycoplasma pneumoniae* or *Chlamydia pneumoniae*)
3. Iron deficiency anemia — chronic, underlying diagnosis
4. Autism Spectrum Disorder, Level 2 — pre-existing, not modified during admission
5. ADHD, Combined Type — pre-existing, not modified during admission
6. PTSD — pre-existing, not modified during admission

## DISCHARGE MEDICATIONS

The following medications are prescribed at time of discharge. This list supersedes all prior medication lists.

| #  | Medication                                            | Dose             | Route     | Frequency            | Duration / Notes                          |
|----|-------------------------------------------------------|------------------|-----------|----------------------|-------------------------------------------|
| 1  | Prednisolone oral solution 15 mg/5 mL                 | 26 mg (~8.7 mL)  | PO        | Once daily AM        | 5 days; begin 03/02/2026                  |
| 2  | Budesonide/Formoterol (Symbicort) 160/4.5 mcg inhaler | 2 puffs          | Inhaled   | BID                  | Ongoing; stepped up from prior 80/4.5 mcg |
| 3  | Albuterol MDI 90 mcg/puff                             | 2 puffs          | Inhaled   | q4-6h PRN            | Ongoing PRN wheeze/dyspnea                |
| 4  | Albuterol 2.5 mg/3 mL neb solution                    | 1 vial (2.5 mg)  | Nebulized | q4h PRN              | Ongoing PRN if MDI insufficient           |
| 5  | Azithromycin 200 mg/5 mL suspension                   | 130 mg (3.25 mL) | PO        | Once daily           | Complete 5-day course through 03/03/2026  |
| 6  | Montelukast 5 mg chewable tablet                      | 5 mg             | PO        | QHS                  | Restarted; continue                       |
| 7  | Risperidone                                           | 1 mg             | PO        | QHS                  | Continue as prescribed                    |
| 8  | Methylphenidate ER 27 mg                              | 27 mg            | PO        | QAM school days      | Resume home regimen                       |
| 9  | Clonidine 0.1 mg                                      | 0.1 mg           | PO        | QHS                  | Resume home regimen                       |
| 10 | Ferrous sulfate 325 mg                                | 325 mg           | PO        | Once daily with food | Iron deficiency anemia                    |
| 11 | PEG 3350 (MiraLAX)                                    | 17 g             | PO        | Daily                | Continue as prior                         |
| 12 | Melatonin                                             | 3 mg             | PO        | QHS                  | Continue OTC as prior                     |

Note to dispensing pharmacy: Rx #744228901 (Symbicort 160/4.5 mcg) submitted to Walgreens #11482, 5348 W. North Ave., Chicago, IL 60639. Prednisolone and azithromycin prescriptions called in to same pharmacy.

## DISCHARGE INSTRUCTIONS — CAREGIVER (Ms. Renee Carter)

### Asthma Action Plan:

**GREEN ZONE (Doing Well):** Symbicort 2 puffs BID every day; no albuterol needed at rest.

**YELLOW ZONE (Caution):** Albuterol 2 puffs with spacer; repeat after 20 minutes if needed; call clinic if more than 2 yellow-zone episodes in one day.

**RED ZONE (Emergency):** Albuterol by nebulizer immediately; if no improvement in 15 minutes or Marcus is working hard to breathe, lips appear bluish, or he cannot speak — call 911 or go to the Emergency Department immediately.

Marcus has had four episodes severe enough to require a breathing tube. This is a life-threatening condition. Do not wait to seek emergency care.

- **Inhaler technique:** Ms. Carter was instructed by Respiratory Therapy on proper spacer technique with Symbicort and albuterol MDI. Return demonstration completed satisfactorily prior to discharge.

- **Avoid known triggers:** Smoke of any kind, strong cleaning products, dusty environments, and mold. Ensure no smoking in the home or vehicle.

- **Azithromycin:** Give the remaining doses at home through 03/03/2026 with food if stomach upset occurs.

- **Prednisolone:** Give in the morning with food for 5 days. Do not stop early.

- **Ferrous sulfate:** Give with food or orange juice. Stools may appear dark — this is normal.
- **Risperidone:** Continue giving 1 mg at bedtime as listed above.

## FOLLOW-UP APPOINTMENTS

### Primary Care Pediatrician (Dr. Anita Sharma, Cook County Pediatrics — or covering PCP)

- Within 1-2 days of discharge — THIS APPOINTMENT IS URGENT. Please call the office today, 03/02/2026, to schedule. Given the severity of this asthma event and the stepping up of controller therapy, prompt primary care evaluation is required.
- Bring this discharge summary and all medication bottles to the appointment.
- Chest radiograph findings should be reviewed with the primary care provider; outpatient follow-up of the right upper lobe opacity is recommended per radiology report.

### Pulmonology (if not already established)

- Consider referral at discretion of PCP given fourth intubation history.

### Psychiatry (Dr. Osei)

- Resume outpatient psychiatry follow-up per established schedule.

### Gastroenterology (Dr. Patel)

- Resume outpatient gastroenterology follow-up; medications affecting IBD management were not modified during this admission.

## PENDING ITEMS AT DISCHARGE

- Outpatient follow-up of right upper lobe radiographic opacity as noted above — clinic to determine whether additional evaluation is warranted in light of patient's history.
- Infliximab remains on clinical hold per prior GI team decision; not addressed during this admission.
- IGRA/TB testing status not evaluated during this admission; defer to outpatient team.
- Growth and nutritional status: PediaSure supplementation initiated; formal dietitian follow-up recommended outpatient.

## CONDITION AT DISCHARGE

Good. Marcus was ambulatory, interactive, and in no respiratory distress at time of discharge. Oxygen saturation 98% on room air. Afebrile. He was discharged to the care of Ms. Renee Carter.

## ATTENDING ATTESTATION

I have reviewed the above discharge summary, hospital course, and medication list. To the best of my knowledge, this summary accurately reflects the clinical care provided during this admission.

Heather Collins, MD

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Electronically signed: 03/02/2026 at 14:37 CST

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